

Applied Scenarios & Ethics Primer

ONLINE ONLY LECTURE NOTES

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Overview

- Types of Applied Scenarios
 - General Approach to Clinical Scenarios
- Ethics
 - Patient Centred Communication
 - Consent and Capacity
 - Fitness to Drive
 - Troubled Colleague / Professionalism
 - End of life care
 - Disclosure of medical error
 - Confidentiality
- Patient Safety/QI → See Online PRIMER with examples
- Truth and Reconciliation in Canada – implications for physicians

Applied / Oral Scenarios

In 2026, your applied / oral scenario will be done in person

- What we know about virtual Applied/Oral Exams (<https://www.royalcollege.ca/rcsite/documents/ibd/internal-medicine-examformat-e>)
 - 6 virtual stations x 20 min each
 - 1 rest station , 6 “work” stations = 7 total
 - May have >1 case per station
 - (eg) read stem 1-2 min, then 7 min scenario, read next stem 1-2 min, 7 min scenario
 - Media provided can include videos or documents (eg ECG or Chest X ray, lists of labs), audio of heart sounds/murmur
 - As of November 2025, we still do not have updated format for 2026 exam with respect to whether there will be standardized patients/actors (as there used to be before 2020 applied exam cancelled due to COVID, and subsequent virtual exams 2021-2025)

Feedback from IMR Attendees



“[Need to practice interruptions causing] stress that is experienced on exam day. I think that on the board exam the examiners do not really give you time to go over all the management including non-pharm treatment. You would get interrupted to move onto the next question.”



“ On the real exam there were many doublet stations where we only had 1 min to read and were pressured to have a differential and plan ready to go right away. ”



“The exam was pretty well all 7.5 min stations with only about 1 min to read the scenario for second part of station. It was much more rapid fire than I anticipated.”



“The real exam has become very rapid fire. Long stem with HPI, P/E and initial Ix then after the prep time it’s really just “what’s your ddx and how would you manage, including further investigations?” They’d show you an EKG or X-ray if that was relevant to your workup. They cram so much into the stations ... that you really can’t stray from the relevant points of the case (in an attempt to pick up extra points for non-pharm things, for example) some of the examiners were quite abrupt.”

Feedback from IMR2024 Attendees



I think I passed that exam because I memorized every single word in your slides. The scenarios need to be more confrontational; it was not uncommon to have a mean examiner, focusing making you miserable in the stations; 40% of my examiners were not nice.



Watching the IMR mock/practice exam videos were extremely helpful - The more the better



I found the outpatient / clinic-based scenarios on the exam to be the most difficult as we don't see much of that in our core IM training



I would suggest practice listening to murmurs to start the case then once the murmur is identified manage a patient with a complication of the murmur. Practice interruptions – (eg) In the middle of a case ask how to differentiate a murmur or how to measure blood pressure.

Types of Applied Scenarios

- Emergency Department / Ward on-call
 - (eg) Patient presenting after travel from Africa with fever and jaundice – workup and manage
- Pre- or Post-op patient
 - (eg) Elective Orthopedic patient seen preop, asked to manage meds, calculate RCRI, manage MINS postop
- Pregnant patient
 - (eg) L&D / ED – hypertension, pre-eclampsia diagnosis, admission, management
- Office Scenario
 - (eg) Patient with diabetes – BP, A1C, Lipids all above target “how would you optimize”?
 - Prompts to cover counselling of patient – driving, eye exams, etc.
- Prior to 2020: Physical exam stations with standardized patients (actors)– examine a patient for a condition or syndrome and then manage complications / counsel patient about condition
- Communication Station (?possibly standardized patients in 2026?)
 - You may be asked how you would counsel a patient on a procedure or treatment (Indications, Contraindications, Risks, Benefits, Risks if you don’t have intervention...)
 - Ethics come up - Know Canadian Laws around medicolegal issues of care: Driving, end of life, consent & Capacity, withdrawal of care, MAID

The format of 2026 exam is not yet updated on RC website – at this time we do not know if standardized patients return @ IM Exam. [Nov 2025]

Clinical Scenarios: General Approach

1. Understand the question

- If provided with a long stem, check the question at end first to tailor your reading
- If your task is unclear from the written stem you are given, **ask to clarify**

“Do you want me to counsel this patient on anticoagulation for atrial fibrillation only or other aspects of Afib management such as rate/rhythm control?”

2. Frame and Markup your “Pink Sheet” [you will be given a piece of paper to take notes on for each scenario]

- Your Differential – do this every time, may get asked by examiner what DDx is
- Management
 - Tests for workup: many provided in stem if it is a management scenario
Be prepared: ECG, CXR, PFTs are fair game for interpretation
 - Non Pharm: SPEDD – Smoking, Pregnancy, Exercise, Driving, Diet
 - Pharm: Acutely ill: ABC MOIF (monitor, oxygen, IV, foley)
C&C (consults and code status)

3) Answer the Question – with new format, you will get pushed towards management

Clinical Scenarios: General Approach

Management PEARLS: ACUTE MANAGEMENT

- Use common sense – in an ED scenario with unstable patient, you must treat first, investigate later (like in real life!)
 - (eg) Afib –BP 70/40, HR 160, patient stuporous
 - “This patient is presenting with acute unstable SVT/AFIB – I would like to move to appropriate resus area of the ER and begin ACLS measures”
 - Cardiovert first, ask questions later
 - (eg) ECG may suggest WPW but if unstable just cardiovert then think about next steps!
 - (eg) ED Tox Scenario – patient starts to seize:
 - Treat Seizure (position, suction available, oxygen, give appropriate order for benzodiazepine)
 - “While the nurse is preparing the lorazepam I would ask for the following stat labs including an immediate capillary glucose and ECG...”
 - UNSURE? SAY WHO YOU WOULD CALL, WHAT YOU WOULD LOOK UP. (Like in real life.). This is a safety scenario – if you can demonstrate knowledge gap and where you would seek answer or help this is a PASS.

Clinical Scenarios – General Approach

Management (continued) : SUBACUTE

“C&C = Consult and Counsel”

- Consult appropriate services
 - “I would consult a hematologist and ICU where plasma exchange is available for this patient with TTP for transfer...”
 - Pregnant Scenario: Don’t forget to consult **OB +/- anesthesia +/- Paeds**
- Counsel
 - I would counsel the patient as to...
 - Medicolegal:** Driving? Work restrictions? Communicable disease / Public Health reporting requirement?
 - Risk of recurrence?** (ex. DVT or HTN in pregnant pt)
 - Consequences of disease** (ex. Afib → Stroke risk)
 - Consequences of treatment** (ex. Afib → anti-arrhythmic side effects), Pregnancy implications if childbearing age.

Ethics

In exams before 2020, there was usually a role-playing communication station. In applied scenario without standardized patient, it is more likely a question of ethics will come up in a management question. (eg) question of consent/capacity in a patient with Jehovah's witness faith refusing blood, driving restrictions post defibrillator or acute stroke

- Patient Centred Communication – A Canadian Healthcare Priority
- Consent & Capacity
- Fitness to Drive
- Troubled Colleague / Professionalism
- End of life care
 - Medical Assistance in Dying (MAID)
 - Withdrawal of care
- Disclosure of adverse events / medical error
 - Patient Safety / Quality Improvement may be asked
- Confidentiality
- Patient Safety / QI

Patient Centred Communication

- Meeting a patient “where they are” and acknowledging the socioeconomic and cultural influences on health (social determinants of health (#sdoh)).
- **FIFE** –ask patients about their **F**eelings (fears) around illness, **I**deas about what has caused illness, how it affects their **F**unctioning, and **E**xpectations of their encounter and treatment
 - (eg) Creating a safe space for gender identity – inviting patients to express gender with preferred pronouns
 - (eg) Being aware of non-visible aspects of culture – such as how emotions are managed, how modesty and physical distancing affect comfort with physical exam
 - (eg) Writing “The patient is noncompliant with dietary recommendations for diabetes” does not acknowledge how food insecurity may make it impossible for patient to comply with your recommendations – screen for SDOH

Consent & Capacity

Scenarios to consider:

- The patient who is refusing life-saving therapy
 - Person living with diabetes and vascular disease refusing above knee amputation
 - Person of Jehovah's Witness faith declining life-saving transfusion
- The family member who asks for a treatment for their cognitively impaired parent.
 - **Variation:** Family member who does not want you to disclose terminal diagnosis to their parent

Consent Requirements

1. Voluntary
2. Informed
3. Capable
4. **Documented in the Chart***
(*CMPA advisor added this)

CMPA Consent and Capacity Module

https://www.cmpa-acpm.ca/serve/docs/ela/goodpracticesguide/pages/communication/Informed_Consent/informed_consent-e.html

Assent ≠ Consent (just because a patient lets you do a procedure (ex. ABG) does not mean that you have their consent.)

Consent: Voluntary

- No compulsion
 - By physician
 - By other 3rd party
 - Police officer
 - Family member

Consent : Informed

- Informed consent components
 - “Reasonable Patient Standard” – Supreme Court
 - What a reasonable patient in the particular patient's position would have expected to hear before consenting.
 - Description of treatment
 - “Material Risks”
 - *“A risk is thus material when a reasonable person in what the physician knows or should know to be the patient's position would be likely to attach significance to the risk or cluster of risks in determining whether or not to undergo the proposed therapy.”*
 - Alternatives to treatment (and risks of alternatives)
- Informed by MD carrying out procedure
 - Can be delegated (ex. to a resident) if delegate has sufficient knowledge and experience to provide explanations

Consent : Capable patient

- An individual who is able to understand
 - the nature and anticipated effect of proposed medical treatment and alternatives
 - Understand and appreciate the consequences of refusing treatment(Source: CMPA Guide to Consent)
- Capacity may fluctuate in hospital

Consent : Incapable patient

If your patient is incapable – identify an appropriate substitute decision maker (SDM) – do not get fooled in a scenario!

Ontario SDM Hierarchy:

1. Power of Attorney for Personal Care
2. Spouse, Common-law spouse* or Partner
3. Parent or adult (>16yrs) children
4. Siblings
5. Any other family member by blood, marriage or adoption
6. Public Guardian & Trustee

If there is a CONFLICT between SDMs → PG&T should make decision in their stead (e.g. if SDMs are son and daughter who completely disagree on treatment)

*Common Law for Health Care

“common law spouse”

- Varies by province
 - 1-3 years of cohabitation
- “marriage-like relationship” (not just roommates!)
- Unlikely to be a question on your exam
 - But unfortunately, it comes up in real life – (eg) dispute between advance directives between a patient’s adult biological children and their common-law spouse (second marriage).

Consent & Capacity

Scenarios to consider:

- The patient who is refusing life-saving therapy
 - Diabetic refusing above knee amputation
 - Jehovah's Witness refusing transfusion
- The family member who asks for an opinion/treatment for their cognitively impaired parent.
 - **Variation:** Family member who does not want you to disclose diagnosis of cancer to their parent.
 - Patient who does not want to hear details of diagnosis, "Doctor do what you think is best"

Consent & Capacity

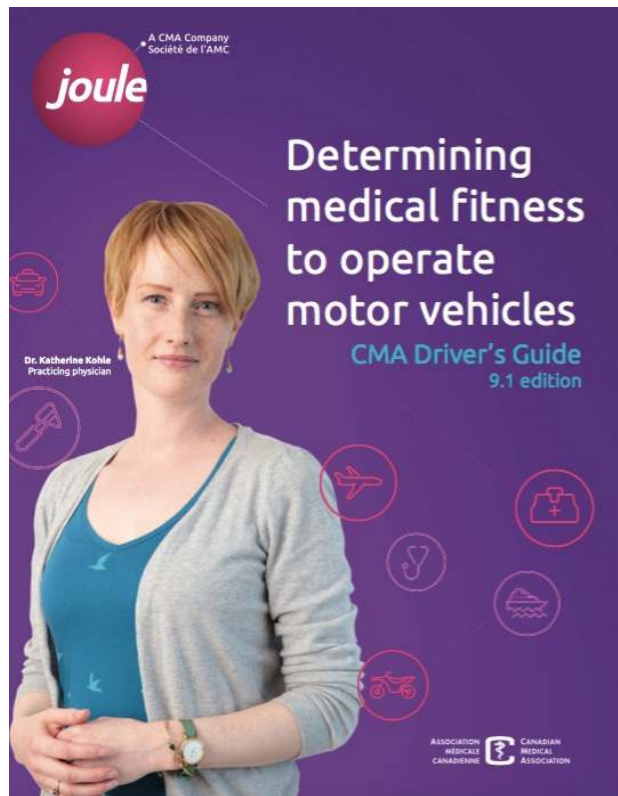
Scenarios to consider:

- Patient who does not want to hear details of diagnosis, “Doctor do what you think is best”

CASE LAW: You can treat a patient without them being fully informed if they waive their autonomy with respect to consent – make sure their decision to waive this is properly informed + capable of course!

[Ontario Health Care and Consent Act doesn't make a 'ruling' on this – it is OK to say in your scenario to patient that you will need to speak to ethicist/CMPA, that you respect their decision and wishes and just need to be sure what your medico-legal obligations are.]

Fitness to Drive



- Available for free to CMA Members
– version 10.0 online
<https://driversguide.ca>
- Also note CCS 2023 published new guidelines on fitness to drive after cardiac illness (incorporated into CMA guide 10.0) – these are reviewed in Cardiology Lecture @ IMR2026 and summaries below.

Fitness to Drive

Consider:

- Truck Driver with visual field impairment
- New Seizure – counsel on driving
- Sleep Apnea – non compliant with CPAP

Fitness to Drive - CMA

- Duty to report
 - Varies province to province including duty to report
 - Alberta, Nova Scotia, Quebec = reporting discretionary
 - In British Columbia, health professionals must report if the patient has a medical condition that makes it dangerous to drive *and* continues to drive after being warned, otherwise is discretionary (e.g. seizures = report).
 - In Ontario (Exam is in Ottawa so default to Ontario rules if unsure) – MDs **must** fill out medical condition report and cannot be legally challenged for doing so:
 - “Medical Condition Report Ontario”
 - Ontario Ministry of Transportation suggests the CMA guide be used by physicians in determining requirements to report

CMA Fitness to Drive:

<https://joule.cma.ca/en/evidence/CMA-drivers-guide.html>

Category		Private Car	Commercial Driver (Truck, Bus)
Cognitive	Hepatic Encephalopathy	No Driving	
	Dementia	UPDATED IN 2019 – SEE NEXT SLIDE	
	Psychiatric – acute psychosis, lack of cooperation w/ treatment or treatment too sedating	No driving	
Stroke	Untreated intracerebral aneurysm	No driving	
	Postop Aneurysm Rx	3 mos	6 mos
	Other stroke, with normal VF, neuro exam	1 month	1 month
Alcohol Dependence	Alcohol Withdrawal Seizures	Seizure free, alcohol free for 6 months – Rehabilitated and Compliant	

CMA Fitness to Drive:

<https://joule.cma.ca/en/evidence/CMA-drivers-guide.html>

Category		Private Car	Commercial Driver (Truck, Bus)
Sleep Problems	OSA – mild or treated (TREATED = at least 4 hrs/d, 70% of last 30 days)	Safe to Drive	
	OSA – mod/severe untreated	No driving	
	Narcolepsy	No driving	
Endocrinology	Diabetes on insulin	May drive if: medic alert worn, no severe hypoglycemia last 6 mos	
Seizure	Single, unprovoked, no epilepsy	3 months Full workup to ID cause	EEG, imaging required – if normal 12 mos
	Epilepsy compliant on medications	6 mos seizure free	5 years seizure free

CMA Fitness to Drive : Dementia

- Diagnosis of dementia alone not sufficient reason to suspend license.
 - Discuss a plan to “retire” from driving with patient
 - Physician is not the one to determine who is fit to drive, but to report clinical findings that raise concern about ability to drive
- It is unlikely that fitness to drive is preserved in moderate dementia (moderate = loss of at least 1 ADL) and it should be discouraged
 - If patient still desires to drive, should be formally assessed
 - Severe dementia is a contraindication to driving
- Where fitness to drive not clear, refer for further functional testing (e.x. OT, DRIVEABLE assessment)
- Ask : would I let a loved one get in the car... would I want a loved one crossing street in front of this individual's car...? If answers are uncertain or “NO” then report and refer for further testing.

CMA Fitness to Drive: Dementia Excerpts

TABLE 2: Consensus recommendations on fitness to drive in patients with dementia*

#	Recommendation	Class of Evidence	Agreement†
1	Dementia often has a direct effect upon fitness to drive, and clinicians should address cognitive compromises that may impact fitness to drive.	C	96.6%
2	Diagnosis of dementia alone is not sufficient to withdraw driving privileges.	A	93.8%
3	Severe dementia is an absolute contraindication to driving.	C	96.6%
4	It is unlikely that safe driving can be maintained in the presence of moderate dementia (e.g., the additional presence of basic ADL impairments) and is to be strongly discouraged. If the patient desires to drive, they should be formally assessed and monitored very carefully.	B	92.4%
5a	People with dementia with progressive loss of 2 or more IADLs due to cognition (but no basic ADL loss) are at higher risk of driving impairment.	A	95.2%
	5b A formal assessment and ongoing monitoring of fitness to drive is recommended in this situation if the patient wishes to continue driving.	B	93.8%

(cont'd)

ia	No in-office test or battery of tests including global cognitive screens (e.g., MMSE, MoCA) has sufficient sensitivity or specificity to be used as a sole determinant of driving ability <i>in all cases</i> .	A	97.2%
	6b However, abnormalities on these tests may indicate a driver at risk who is in need of further assessment.	B	95.9%
	6c Substantially impaired scores, which are typically associated with moderate to severe dementia, may preclude safe driving.	C	84.1%
	6d If concerns or uncertainty still exist, a specialist opinion should be sought.	C	81.4%
	Patients with dementia who are deemed fit to continue driving should be re-evaluated every 6 to 12 months or sooner, if indicated.	B	93.1%
1a	Any clinician who has concerns but is uncertain whether a patient's cognitive problems may adversely affect driving, should refer the patient for a functional driving assessment, either through an occupational therapy evaluation or directly to the licensing authority.	C	85.5%
	8b If there are clear aspects of the history, physical examination and cognitive examination that place the patient and public at high risk for crash or impairment, the patient and informant/caregiver should be advised not to drive, and this conversation (including date and participants) should be documented in the clinical record.	C	96.6%
	8c Clinicians should be aware of the legal reporting requirements in their jurisdiction, mindful of their professional ethical imperatives, and strive to ensure that mechanisms to remove unsafe drivers from the road are sensitive, timely and effective.	C	85.5%

CMA Fitness to Drive: Dementia Excerpts

9a	Caregivers are able to predict driving safety more accurately than can the patients themselves, although in some circumstances, the caregivers may have a vested interest in preserving the patient's autonomy beyond a safe window ...	C	82.1%
	9b... Hence, caregiver concern about driving impairment should be taken seriously ...	B	96.6%

(cont'd)

Dementia

4

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	9c... and the possibility of a conflict of interest in preserving driving autonomy must be taken into consideration if such caregiver concern is absent.	C	92.4%
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10	Medical comorbidities, physical frailty and the use of multiple medications are also factors that must be taken into consideration when assessing fitness to drive.	C	93.1%
11	We recommend a formal evaluation if behavioural disturbances (e.g., agitation, personality change, psychosis) are concerning for interfering with safe driving.	C	85.5%
12	Patients with prominent language impairment, e.g., primary progressive aphasia or other aphasia in the context of dementia, cannot be adequately screened with typical language-based tests and require a specialized assessment possibly from a speech therapist or neuropsychologist, functional assessment (IADLs, ADLs) and/or a formal driving assessment.	C	84.1%
13	As with many disabling progressive diseases that lead to driving cessation, conversation regarding eventual retirement from driving should be held as early as possible.	C	89.0%
14a	Driving cessation has been associated with social isolation, depression and other adverse health outcomes.	C	90.3%
	14b Therefore, after a person with dementia has stopped driving, it is important to monitor for these problems longitudinally.	C	87.6%

CMA Fitness to Drive:

<https://joule.cma.ca/en/evidence/CMA-drivers-guide.html>

Category		Private Car	Commercial Driver (Truck, Bus)
VISION	Visual Acuity	≥20/50 Corrected both eyes open	≥20/30 Corrected both eyes open
	Visual Fields	120 continuous degrees along horizontal meridian 15 continuous degrees above and below fixation	120 continuous degrees along horizontal meridian 20 continuous degrees above and below fixation



CCS 2023 Fitness to Drive Guidelines

Category		Private Car	Commercial Driver (Truck, Bus)
Coronary Artery Disease*	STEMI/NSTEMI with LVEF >40%	2 weeks post d/c	1 months post d/c
	STEMI/NSTEMI with LVEF ≤ 40%	1 month post d/c	3 months post d/c
	STEMI/NSTEMI with no PCI performed	1 month post d/c	3 months post d/c
	UA (ACS without MI)	48h w/ PCI 7 d w/o PCI	7 d w/ PCI 1 month w/o PCI
	PCI in non-ACS context	48h	48h
	Asymptomatic CAD, stable angina	OK to drive	OK to drive
	CABG	1 month post d/c	3 months post d/c

Category		Private Car	Commercial Driver (Truck, Bus)
Heart Failure**	NYHA I	OK to drive	LVEF ≥ 30% OK
	NYHA II	OK to drive	LVEF ≥ 30% OK
	NYHA III	OK to drive	NO DRIVING
	NYHA IV	NO DRIVING	NO DRIVING
	Home inotropes	NO DRIVING	NO DRIVING
	LVAD	2 months after implant if NYHA 1-2	NO DRIVING
	Heart transplant	6 weeks after discharge + NYHA 1-2 + stable immunosuppression + followed annually	6 months post discharge + NYHA 1 + LVEF ≥ 50% + followed annually

*Updates focus on LVEF post MI, regardless of mechanism

**Updates provide new recommendations for LVAD/TxP



CCS 2023 Fitness to Drive Guidelines

Category		Private Car	Commercial Driver (Truck, Bus)
Valvular Heart Disease	Medically treated AS, AI, MS, MR, TR	AS: OK if NYHA 1-2 MS, MR, AI, TR: OK if NYHA 1-3	Only OK if in the lowest risk group: NYHA 1, no syncope, LVEF $\geq$ 50% AS, AI: as above MR: also no pHTN or systemic embolism MS: no LVEF requirement TR: no RV dysfunction, symptomatic RV failure, RV arrhythmias
	Post TAVI or SAVR for AS, AR	1 month if stable QRS duration, no high grade AV block (2 nd type II or 3 rd) if no PPM and NYHA I-III AR requires as above + LVEF $\geq$ 50%	3 month if stable QRS duration, no high grade AV block (2 nd type II or 3 rd) if no PPM and NYHA I AR requires as above + LVEF $\geq$ 50%
	Post TEER for MR or TR	48h post d/c if NYHA I-III	1 month post d/c if NYHA I and LVEF $\geq$ 50%
	Post balloon valvuloplasty for MS	48h post d/c if NYHA I-III	1 month post d/c if NYHA I
	Post TMVR, TTVR or surgical valve replacement	1 month post d/c if NYHA I-III	3 months post d/c if NYHA I and LVEF $\geq$ 50%

Category		Private Car	Commercial Driver (Truck, Bus)	Arrhythmia	Private Car	Commercial Driver (Truck, Bus)
Pace-maker	Impaired LOC or high grade AV block	1 week post implant		Sinus node dysfunction	No impaired LOC: OK to drive Impaired LOC/symptomatic pauses: NO DRIVING until PPM	
	No impairment in LOC/high grade AV block	OK to drive		First degree, Wenckebach, isolated LBBB, LAFB, LPFB, RBBB, bifasicular block	OK to drive unless history of impaired LOC	
	Generator change (also applies for ICD private driving)	OK to drive		Second degree type II, alternating BBB, 3 rd degree AVB	NO DRIVING until PPM Exception: congenital 3 rd AVB (only restricted if impaired LOC)	
	Lead upgrade/revision (also applies for ICD private driving)	1 week post implant if history of impaired LOC/high grade AV block Otherwise OK to drive		SVT/AF/AFL	No impaired LOC: OK to drive Impaired LOC: NO DRIVING until treated	
				VF (no reversible causes)	3 months	NO DRIVING
ICD	Primary prophylaxis	1 week post implant	NO DRIVING	VT/VF due to reversible cause (MI, drug, etc.)	NO DRIVING until underlying cause treated	
	Secondary prophylaxis, impaired LOC	3 month		Hemodynamically unstable VT or impaired LOC	3 months	NO DRIVING
	Secondary prophylaxis, no impaired LOC	1 week		VT with structural heart disease and no impaired LOC, no ICD	3 months	
				VT with structurally normal heart (idiopathic VT)	1 week if well controlled	



CCS 2023 Fitness to Drive Guidelines

Category		Private Car	Commercial Driver (Truck, Bus)
ICD therapy	ICD shock or therapy, impaired LOC or disabling	3 months	NO DRIVING
	ICD shock or therapy, NO impaired LOC or disabling	1 week	
	Inappropriate ICD therapies	No restriction	
	Electrical storm (3 or more VT/VF events in 24h)	3-6 months after event (expert re-evaluation)	
Hyper-trophic cardio-myopathy	No high risk features	OK to drive	
	High risk features**	Asymptomatic: OK to drive	NO DRIVING
		Syncope: 3 months	
	Prior sustained ventricular arrhythmia	3 months	

Category		Private Car	Commercial Driver (Truck, Bus)
Syncope	Single/recurrent vasovagal syncope	OK to drive	
	Reversible cause (orthostatic, dehydration) or avoidable trigger (micturition)	1 week	1 month
	Single unexplained syncope	1 week	12 months
	Recurrent unexplained syncope	3 months	12 months
	Tachy/brady/device syncope	Refer to respective arrhythmia or device recommendation	

**High risk features: think ICD recommendations i.e. wall thickness >30mm, unexplained syncope, apical aneurysm, LVEF <50%),

Fitness to Drive

Consider:

- Truck Driver with visual field impairment
- New Seizure – counsel on driving
- Sleep Apnea – non compliant with CPAP, history collision

Approach:

- Discuss medical diagnosis, ensure they understand it
- Ask leading questions – how might this impact your work? What are risks if the seizure reoccurs / daytime sleepiness affects job...?
- Ask if they drive
 - Disclose your medico-legal obligation to report medical condition to Ministry of Transportation
 - Discuss the law in clear terms: they must not drive for x months, until the MOT reinstates license following appropriate medical follow-up/clearance
 - If you forget the amount of time, state your legal obligation to inform the MOT and advise them that you will contact them with the specific information at a later appointment
- Offer Support, appropriate referrals (ex. For sleep study), and plan to treat medical condition

Counsel a Troubled Colleague

- You are working in a group providing hospitalist care in a community hospital. One member of your group has caught the ire of the group – patients state they are not rounded on, he has canceled shifts at the last minute.
- Nurses state he is prone now to outbursts of anger
- One morning you note he has alcohol on his breath

The Troubled Colleague

- Know your responsibilities
 - Hospital by-laws, medical staff policies
 - College of Physicians protocols
 - CPSO policy – available at www.php.oma.org
- Know your limits
 - In a scenario, if you are asked to address this with a colleague, you may need to escalate to discussing w/ Chief of Staff and tie things up there.
- Sample Script:
 - Review the Facts (“I understand you did x,y,z – can you tell me what happened?”)
 - Review the Code of Conduct (hospital by-laws)
 - Find common ground
 - Probe for cause of troubles
 - “Can You help me understand what happened on the ward that might have prompted your anger?”
 - Offer support (Physician Health Program)
 - Discuss your obligations – “As Division Head I will document our meeting and provide you with a copy... I have to forward this to the medical advisory council...”
 - Discuss their obligations – as per the Code of Conduct

SOURCE: CPSO 2008 “Guidebook for managing disruptive physician behaviour”

<http://php.oma.org/PDF%20files/Guides/CPSOGuidebook.pdf>

End of Life Scenario

Consider:

- Wife asks you to assist in providing her husband, 62yM non-verbal with advanced primary progressive MS with a “dignified death” (Medical Assistance in Dying)
- A 58yM is in a persistent vegetative state in the ICU, ventilated. No evidence of meaningful recovery after 4 weeks of full medical therapy. Discuss treatment options with family including withdrawal of life sustaining therapy.

End of Life Scenario: Medical Assistance in Dying

- February 2015 – Superior Court of Canada ruled in Carter v. Canada, that the prohibition of assisted suicide was contrary to Canadian Charter of Rights and Freedoms
- June 2016 – Royal Assent for Act to amend Criminal Code to allow Medical Assistance in Dying
- **March 2021 – Bill C7 Amendment approved by Senate**
 - Bill is still not inclusive of 1^o mental illness – decision by Federal Government not expected until March 2027.
- **Oct 30, 2024 – In QUEBEC ONLY – Advanced requests**

ADVANCE REQUESTS.

(eg) Scenario for someone living with dementia who is currently capable to say “If I become incapable and am dependent for toileting or do not know names of my spouse and children I want MAID”.

The Government still needs to work out logistics and **this is not currently legislated/permitted MAID practice in Canada (other than QC, Bill 11 as of Oct 30, 2024).** This is not likely to be a scenario as the law across country (federal criminal code) still does not allow for this practice.

- 2 options
 - MD or NP directly administers a substance that causes death
 - MD or NP provides a drug that person self-administers

****HIGH YIELD****

Medical Assistance In Dying – the law changed March, 2021

Eligibility (must meet ALL of the following...)

1. Be eligible for health services funded by government (i.e. have a health card)
2. Be ≥ 18 years old and **have decision making capability** 
3. Have a **grievous and irremediable*** medical condition
 - “Have a serious and incurable illness, disease or disability”
 - Excludes Mental Illness... ****pending further review by Federal government, due March 2027****
 - <https://www.camh.ca/en/camh-news-and-stories/maid-and-mental-illness-fags> has a nice summary
 - Mental Illness includes conditions primarily in domain of psychiatry (ie depression), but does NOT include neurocognitive or neurodevelopmental disorders.
4. Make a voluntary request for MAID, free from outside pressure or influence
5. Provide informed consent to receive MAID

Pt must be capable: Cannot be in delirium, advanced dementia

*Grievous and irremediable definition

- Does not need to be a fatal or terminal condition
- Must be a serious illness, disease, disability in an advanced, **irreversible state**

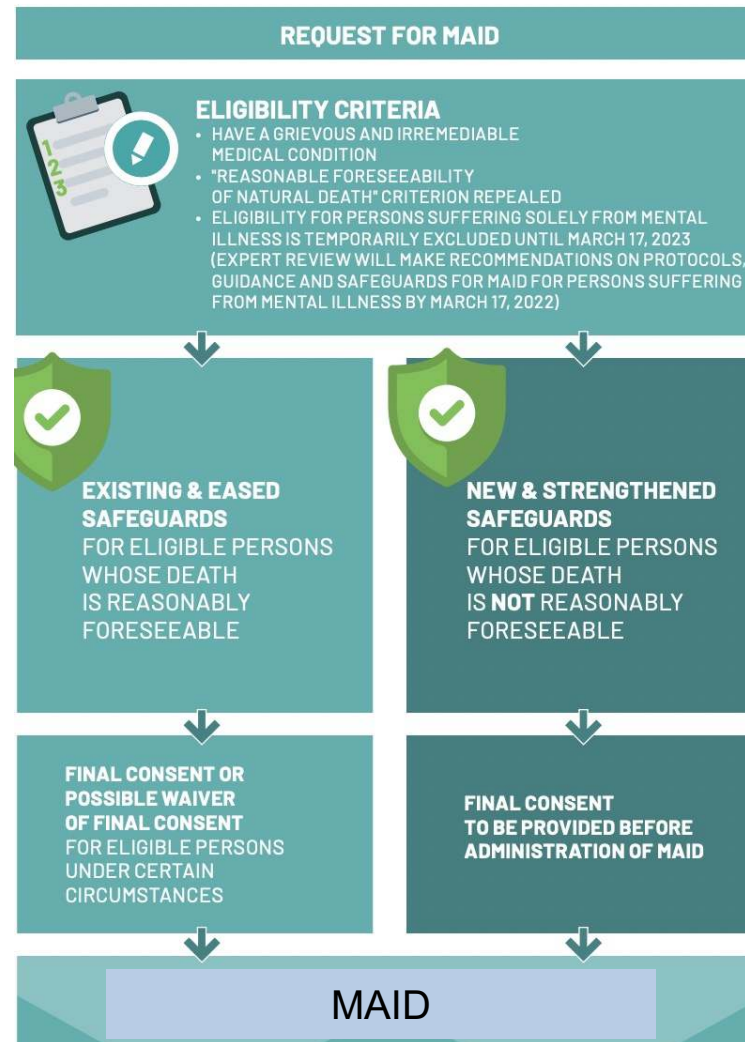
As of March 2021 – Supreme Court removed requirement that death is reasonably foreseeable, 39

NEW MAID LAW
CAME INTO FORCE ON MARCH 17, 2021

**Stream 1: Death is reasonably
Foreseeable**

- Request in writing, signed by independent witness
- 2 independent MD/NPs must confirm all eligibility criteria met
- Person must be given opportunity to withdraw consent, and confirm consent expressly before receiving MAID

**10 day reflection period is now
REMOVED for those whose death is
reasonably foreseeable**



Stream 2: Death is NOT foreseeable

- Request in writing, signed by independent witness
- 2 independent MD/NPs must confirm all eligibility criteria met AND consult with expert in the medical condition (if MD/NP is unfamiliar with it)
- The person must be informed of means to alleviate suffering and be offered consults with experts in this (incl. counselling, pall care...)
- **The eligibility assessments must take at least 90d**, but this period can be shortened if pt about to lose capacity
- Person must be given opportunity to withdraw consent, and confirm consent expressly before receiving MAID

MAID under Bill C7:

Practical Examples of what has changed



A 75yF diagnosed with Dementia. Is capable at this time. Now under Bill C7 can request MAID **on a certain date** in the near future and give consent for procedure; recognizing that they may lose capacity for consent by date of procedure.



A 48yM has metastatic colorectal cancer. He has a malignant bowel obstruction and is in severe pain, unable to eat and is obstipated with constant vomiting. He is no longer required wait 10 days from requesting MAID to completion of the procedure.

MAID: Changes to Final Consent Requirement

- Pt does not have to provide consent immediately before provision of MAID if :
 - Pt has been assessed and approved for MAID
 - Pt was at risk of losing decision making capability prior to receiving MAID and was made aware of that risk
 - Person makes arrangement in writing with their practitioner to waive final consent, and according to which their practitioner will provide MAID on their preferred date if they have lost capacity (*“Audrey’s amendment”*)

MAID Primer in Ontario

- <https://www.cpso.on.ca/Physicians/Policies-Guidance/Policies/Medical-Assistance-in-Dying> [Updated: Dec, 2023]
 - CPSO requires that you offer palliative care (and best practices would support this!)

If you conscientiously object to MAID you **must** refer them to someone else

- Must also provide them with information about the process
- Must not 'abandon' the patient – provide effective referral
- Must not express personal moral judgements about the choice of MAID
- MOHLTC in Ontario has hotline for this to help you find provider in your area- 1-844-243-5880

Withdrawal of Care & Withholding of CPR

Withdrawal of care *against wishes of patient/SDM* is not permissible through the Supreme Court (*Cuthbertson v Rasouli* 2014)

- Ruled that **withdrawal of treatment** requires the consent of patient / SDM
- MDs can still appeal via Consent & Capacity board if has evidence SDM not acting reasonably or in patient's best/prior stated wishes

BUT ... Withholding of life support - permissible with conditions per Ontario Superior Court (*Wawrzyniak v. Livingstone*, Sept 2019)

- the Court concluded that the writing of a no-CPR order and withholding of CPR do not fall within the meaning of "treatment" in the *Health Care Consent Act, 1996*, S.O. 1996, c. 2, Sched. A. As such, consent is not required prior to withholding CPR and physicians are only obliged to provide CPR in accordance with the standard of care.

Know your College Policies

- Withdrawal of care or Withholding of life support
 - CPSO: updated policy 2019 ("planning for and providing quality end of life care")
 - Consent is not required to write a DNR Order BUT
 - » Must inform patient / SDM of why the order is written
 - » If patient/SDM disagree and insist on 'yes' CPR order → conflict resolution process → Physicians must by default perform resuscitative efforts (which may include CPR based upon standard of care of profession] while conflict resolution underway]
 - Conflict Resolution measures
 - » Medical ethics consult, access to mediation or arbitration committees
 - » Second opinion from physician with expertise in area
 - » <https://www.cpso.on.ca/Physicians/Policies-Guidance/Policies/Planning-for-and-Providing-Quality-End-of-Life-Care>

End of Life Scenario

Consider:

- Wife asks you to assist in providing her husband, 62yM non-verbal with advanced primary progressive MS with a “dignified death”

How to handle this ethical quagmire:

- 1) Make it clear to wife that husband must be capable and consent to all treatments. You will have to assess his capacity (non verbal does not mean incapable).
- 2) Empathize. You must perceive that he is suffering. How is he suffering? If it is pain we can refer to palliative care.
- 3) You may mention that withholding life sustaining therapies may be part of palliative strategy eg no enteral feeding when he is unable to eat/swallow, no antibiotics for aspiration pneumonia; but need to assess his CAPACITY first.

End of Life Scenario

Consider:

- A 58yM is in a persistent vegetative state in the ICU, ventilated. No evidence of meaningful recovery after 4 weeks of full medical therapy. Discuss treatment options with family including withdrawal of life sustaining therapy.

Approach to this scenario:

This is not an 'euthanasia scenario'. Family member will have some initial opposition in exam (eg. "we just can't give up on dad!") but if you approach it as:

- Did your dad have any prior conversations with you about ... (is there a living will?)... for purposes of exam they will likely say something like "oh he was so independent he would hate to be paralyzed like this and dependent on machines..."
- "Our treatments may be causing pain and suffering, and we cannot expect meaningful recovery to allow him to be conscious and engage with us. While we want to hope for him, we have to respect his wishes and not prolong his suffering in this state..."

Disclosure of Medical Error

You are following a 63yM for COPD in the community. He presents with supraclavicular and cervical lymph node swelling, and a lung nodule. A biopsy is done; he is diagnosed with metastatic papillary thyroid cancer.

On review of his chart you discover a CT thorax report from 12 months prior which incidentally identified a 3cm Left thyroid nodule with microcalcifications. The radiologist recommended clinical correlation and ultrasound guided biopsy in the body of the report.

Canadian Disclosure Guidelines

Available: www.patientsafetyinstitute.ca, also

<https://www.cmpa-acpm.ca/en/education-events/good-practices/physician-patient/disclosure-of-patient-safety-incidents>

✓ What patients want to hear:

- ☐ The facts of what happened
- ☐ What will be done to minimize harm going forward
- ☐ What will be done to prevent similar adverse events in the future
- ☐ I'm / we're sorry

Confidentiality

- *Personal health information*
 - Info related to physical/mental health
 - Health care plan
 - Medical record, eligibility of patient for health services/insurance info
 - Information about a patient's SDM
- *Permitted Disclosure – try to get consent for disclosure first*
 - Emergency situation
 - Significant risk to other individuals
- *Mandatory Disclosure*
 - Patient poses immediate risk to self or others
 - Motor vehicle risk (i.e. seizure while driving)
 - Risk of harm to child or underaged
 - Court Summons/Order or Subpoena

<http://www.cpsso.on.ca/uploadedFiles/policies/policies/policyitems/Confidentiality.pdf>

Mr Smith is admitted following a seizure while driving. Police Officers in the ED are concerned because, "His breath smells of alcohol." They ask you to add a serum alcohol level to his bloodwork. Mr. Smith is post-ictal and incapable.

What are your obligations to Mr. Smith, to the Police, and to Society?

= report to ministry of transport (in Ontario)

*= **do not disclose to Police** without warrant equivalent*

QI Primer – Check out our QI Primer online!

Guidelines / Resources	Recorded Lectures	Lecture Slides
Review Videos	Applied Exam Oral Scenario Demos	Self-Tests
Physical Exam Handouts	FAQs and Lecture Updates	Sample MCQs and ECGs

1) Use the same e-mail address that you used to register for this course, or you will not receive feedback on your responses.

2) You have to complete the whole test, and then your response will be "submitted". If you then click "VIEW YOUR SCORE" you will get your score for multiple choice questions plus explanations for each question.

Note for questions where you have to Free Text your answer (e.g. differential diagnosis of Chest Xray) the form is unable to evaluate your free text for accuracy so be sure to go over our answer key.

2022 QUALITY IMPROVEMENT PRIMER

2022 Quality Improvement Primer

Quality Improvement and Patient Safety (QI or QI&PS) is a Royal College objective for all Canadian Physicians. Most residency programs include some formal QI training. The basics are enough to work through an oral scenario or answer any multiple choice questions that arise. This Primer is designed for those without training in QI or who want a refresher on the basics.

There are a myriad of resources available to physicians to support QI in the workplace. You



Resources for QI:

Patientsafetyinstitute.ca →
Toolkits → Framework for QI

Health Quality Ontario
Hqontario.ca

<http://www.hqontario.ca/portals/0/Documents/qi/qi-quality-improve-guide-2012-en.pdf>

Truth and Reconciliation



The Indigenous peoples of Canada were subject to decades of abuse by the Government of Canada through the "Indian Residential Schools".

IRS date from 1870-1996

Over 150,000 Indigenous children were taken from their families

90-100% suffered severe physical, emotional, and sexual abuse.

There was a very high mortality rate of IRS - both directly while in care, and due to consequent physical and mental illness in survivors

The Truth and Reconciliation Commission of Canada was established in 2007 and in 2015 made 94 Calls to Action. Below are the seven calls to action under Health Care (numbers 18-23):

TRC Healthcare Calls to Action



18. We call upon the federal, provincial, territorial, and Aboriginal governments to acknowledge that the current state of Aboriginal health in Canada is a direct result of previous Canadian government policies, including residential schools, and to recognize and implement the health-care rights of Aboriginal people as identified in international law, constitutional law, and under the Treaties.

EXAMPLES OF INEQUITIES BROUGHT ON BY IRS PROGRAM (TRC 2015):

- **Infant mortality rate** of First Nations and Inuit children range from **1.7 – 4.0x** non Aboriginal rate
- First Nations people >45 have **2X** rate of **diabetes**
- First Nations peoples **6X more likely to suffer alcohol-related death**, 3X more likely to suffer drug related death
- Suicide rate of First Nations communities >2X that of total Canadian population
 - 5-6X more likely for youth

19. We call upon the federal government, in consultation with Aboriginal peoples, to establish measurable goals to identify and close the gaps in health outcomes between Aboriginal and non-Aboriginal communities, and to publish annual progress reports and assess long term trends. Such efforts would focus on indicators such as: infant mortality, maternal health, suicide, mental health, addictions, life expectancy, birth rates, infant and child health issues, chronic diseases, illness and injury incidence, and the availability of appropriate health services.

20. In order to address the jurisdictional disputes concerning Aboriginal people who do not reside on reserves, we call upon the federal government to recognize, respect, and address the distinct health needs of the Métis, Inuit, and off-reserve Aboriginal peoples.

TRC Healthcare Calls to Action



21. We call upon the federal government to provide sustainable funding for existing and new Aboriginal healing centres to address the physical, mental, emotional, and spiritual harms caused by residential schools, and to ensure that the funding of healing centres in Nunavut and the Northwest Territories is a priority.

22. We call upon those who can effect change within the Canadian health-care system to **recognize the value of Aboriginal healing practices and use them in the treatment of Aboriginal patients in collaboration with Aboriginal healers and Elders where requested by Aboriginal patients.**

23. We call upon all levels of government to:

- i. Increase the number of Aboriginal professionals working in the health-care field.
- ii. Ensure the retention of Aboriginal health-care providers in Aboriginal communities.
- iii. Provide cultural competency training for all healthcare professionals.

24. We call upon medical and nursing schools in Canada to require all students to take a course dealing with Aboriginal health issues, including the history and legacy of residential schools, the United Nations Declaration on the Rights of Indigenous Peoples, Treaties and Aboriginal rights, and Indigenous teachings and practices. This will require skills-based training in intercultural competency, conflict resolution, human rights, and anti-racism.

Education and Support



Physicians should learn more per TRC calls to action.

INTERESTED IN READING MORE AS AN INTERNIST?

THIS WEBSITE THROUGH THE NORTHERN ONTARIO SCHOOL OF MEDICINE HAS A DECENT COMPENDIUM
OF OPEN-ACCESS REFERENCES

<https://www.nosm.ca/education/rehabilitation-studies/resources/indigenous-health-learning-resources>

INDIGENOUS MENTAL HEALTH SUPPORTS

INDIGENOUS MENTAL HEALTH SUPPORTS

The National Indigenous Residential School Crisis Line 24-hour crisis support line [1-866-925-4419](tel:1-866-925-4419) for
Residential school students and their families

Hope for Wellness support for any First Nation, Metis or Inuit [1-855-242-3310](tel:1-855-242-3310) /online chat at
<https://www.hopeforwellness.ca/>